

AC7988 · READ™ Patient Education · Copy Deck with Reference Markup

Client	Alcon LATAM
Brand	READ
Job Code	AC7988
Project	READ Patient Education Brochure
Document	D5 copy deck — reference markup

Version History

Date	Version	Edits / Notes
July 30, 2026	D1	Copy deck creation
August 7, 2026	D2	Revised deck
August 13, 2026	D3	Revised deck after client review
August 19, 2026	D4	Client feedback + updates
August 28, 2026	D5	Copy cut down
September 8, 2026	D5 + refs	Reference markup against REF-15633, REF-20457, REF-29185
September 8, 2026	D5 + refs 2	Added sources 4–9 to support previously unreferenced FAQ/lifestyle lines

How this markup works

Each claim in the D5 copy was checked against the three provided source PDFs. Where a source supports the wording (or the same medical meaning), a superscript citation was added. Reference numbers follow first appearance in the copy. Claims with no matching support in the three PDFs are left unmarked in the copy and flagged in the support grid at the end of this document.

Markup	Meaning
Superscript ¹	REF-29185 — National Eye Institute (NIH). Presbyopia. Last updated December 4, 2024.
Superscript ²	REF-15633 — Cleveland Clinic. Presbyopia. Last reviewed June 8, 2020.
Superscript ³	REF-20457 — Gatinel D, Azar DT, Dumas L, Malet J. J Refract Surg. 2014;30(10):708-715.

Superscript ⁴	MedlinePlus. LASIK eye surgery. Updated January 27, 2026.
Superscript ⁵	American Academy of Ophthalmology. LASIK — Laser Eye Surgery.
Superscript ⁶	American Academy of Ophthalmology. Laser Surgery Recovery. Published May 25, 2023.
Superscript ⁷	Mayo Clinic. LASIK eye surgery.
Superscript ⁸	Wang C, et al. Clin Exp Ophthalmol. 2019;47(7):909-917.
Superscript ⁹	Courtin R, et al. J Refract Surg. 2016;32(11):734-741.

Citation style in copy: Presbyopia is a normal part of aging.^{1,2} *Combined superscripts mean more than one source independently supports the same claim.*

[COPY DECK]

[Header]

See Life Up Close Again

Learn more about READ™, presbyopia laser vision correction^{2,3}

[Subhead]

Is My Vision Changing?

[Body]

If you have difficulty with close-up tasks, such as reading a book, checking your phone, or looking at a restaurant menu, it may be a sign of presbyopia.^{1,2}

Presbyopia is a normal part of aging.^{1,2} Over time, the natural lens in your eye becomes less flexible, and stops focusing light correctly on the retina, which makes nearby objects look blurry.^{1,2}

Common Symptoms Include:

- Blurred vision up close^{1,2}
- Holding reading material farther away to see it clearly^{1,2}
- Tired or strained eyes during close-up tasks^{1,2}
- Headaches after reading or other close-up work^{1,2}

[Subhead]

How is Presbyopia Managed?

[Body]

Your eye care professional may suggest any of the following to manage your vision:^{1,2}

- Reading or prescription glasses^{1,2}
- Progressive lenses²
- Contact lenses^{1,2}

- Lens replacement procedures²
- Laser vision correction^{2,3}

A modern approach to laser vision correction that may be recommended for presbyopia is READ™, which stands for Refractive Enhanced Adjusted Distance.^{2,3}

MLR note — product name expansion

The scientific sources support laser vision correction as a presbyopia option (2) and aspheric/Custom Q-type corneal ablation as a proposed surgical approach to compensate for presbyopia (3). The brand name READ™ and the expansion “Refractive Enhanced Adjusted Distance” are product nomenclature and are not named in the three source PDFs.

[Subhead]

What is READ™?

[Body]

The READ™ procedure is designed to address presbyopia³ and optimize each eye so they can continue working together to support vision across near, intermediate, and far distances^{2,3} – with less dependence on glasses or contact lenses.²

MLR note — mechanism vs. product claim

Source 3 supports modifying corneal asphericity / inducing negative spherical aberration to extend depth of focus as a surgical approach to compensate for presbyopia (including Alcon Custom Q aspheric profiles). Source 2 supports binocular strategies (monovision / modified monovision / multifocal optics) covering near, intermediate, and distance, and lists laser refractive surgery among options that can reduce the need to rely only on spectacles or contact lenses. The three PDFs do not report READ™-specific clinical outcomes.

[Visual]

How READ™ Works

[Visual placeholder — no body copy to reference in D5]

[Subhead]

Could READ™ Be Right for Me?

[Body copy]

Get back to the activities you love — comfortably and clearly²

- Read with ease without straining or searching for your glasses.²
- Stay connected and scroll through your day with clarity.^{5,8,9}
- Pursue your passions and hobbies with greater comfort.²
- Greater freedom from glasses and reclaim the freedom of natural vision.²

READ™ may be an option if you have presbyopia and are interested in reducing your dependence on glasses.^{2,3}

MLR note — lifestyle lines

“Stay connected and scroll through your day with clarity” is still promotional phrasing. Related support: 5 (LASIK allows most everyday tasks without lenses); 8 (64.3% of smartphone-owning presbyopes reported difficulty using phones; worse near acuity was associated with more difficulty); 9 (Custom-Q + monovision: 83% achieved J3 or better binocular uncorrected near acuity). Source 2 covers hobbies/reading, not phones specifically.

[Subhead]**Frequently Asked Questions****1. What happens if I decide to have the procedure?**

Your eye care professional may perform an eye exam, review your health history and lifestyle to confirm if you’re a candidate for READ™. ^{1,2}

The procedure is quick, usually a few minutes per eye. ^{4,7}

MLR note — procedure time wording

4: “LASIK is an outpatient surgical procedure. It takes 10 to 15 minutes to perform for each eye.” 7: “LASIK surgery is usually completed in 30 minutes or less.” The copy’s “a few minutes per eye” is a simplification of those times; consider aligning to “about 10 to 15 minutes per eye” if MLR wants a closer match.

2. Will I still need glasses after READ™?

Individual results may vary but READ™ can reduce your dependence on glasses or contact lenses. ²

MLR note — results language

“Individual results may vary” is a disclaimer, not a cited claim. Source 2 supports that refractive surgery is among options used so patients are not limited to glasses or contacts for near work. The three PDFs do not contain READ™-specific spectacle-independence rates.

3. Will I experience discomfort?

Numbing eye drops are typically used to help you experience little to no discomfort. After the procedure, your doctor may recommend or prescribe medication or eye drops to help manage any temporary discomfort.

Individual results may vary. ^{4,5,7}

MLR note — comfort language

4: “The only anesthetic used is eye drops that numb the surface of your eye.” The surgeon “may prescribe mild pain medicine”; postoperative “eye drops to help prevent infection and inflammation.” 5: “Your eye will be numbed with eye drops.” Afterward, eyes “may feel scratchy or... burning”; “special eye drops to reduce dryness and help your eye heal.” 7: numbing drops during the procedure; “You generally will experience little pain”; “pain medicine or eye drops to keep you comfortable for several hours after the procedure.” “Little to no discomfort” during the procedure is supported; some sources describe moderate scratchiness after the anesthetic wears off.

4. How soon can I get back to my routine?

Many people are able to return to normal activities the next day, while others may experience temporary side effects. Each person’s recovery after laser eye surgery is different. ^{4,6,7}

MLR note — recovery (LASIK, not READ-specific)

6: “In most cases, you can return to work, as well as drive, the day after the procedure.” Temporary blurry/changing vision, dry eyes, glare or halos can last weeks or months. PRK recovery is slower. 4: blurriness “will improve by the next day”; vision usually stabilizes in a few days, but may take 3 to 6 months for some. 7 (in-depth): “Discomfort after surgery is minimal, and vision recovery usually takes place in 1 to 2 days.” These are general LASIK statements, not READ™ outcomes.

[General Note]

*The information provided in this FAQ is intended for general educational and informational purposes only and is not intended to replace the professional judgment, clinical assessment, or individualized recommendations of a qualified eye care professional. Please consult with and follow the recommendations of your eye care professional regarding your eye care or treatment.*¹

[CTA]

See What’s Possible.

Talk with your eye care professional to see if READ™ is right for you.²

[MICE]

References

No.	Alcon REF ID	Citation
1	REF-29185	National Eye Institute (National Institutes of Health). Presbyopia. National Eye Institute website. Last updated December 4, 2024. Available at: https://www.nei.nih.gov/learn-about-eye-health/eye-conditions-and-diseases/presbyopia
2	REF-15633	Cleveland Clinic. Presbyopia. Cleveland Clinic website. Last reviewed June 8, 2020. Available at: https://my.clevelandclinic.org/health/diseases/8577-presbyopia
3	REF-20457	Gatinel D, Azar DT, Dumas L, Malet J. Effect of anterior corneal surface asphericity modification on fourth-order Zernike spherical aberrations. J Refract Surg. 2014;30(10):708-715. doi:10.3928/1081597X-20140903-10
4	Assign REF ID	MedlinePlus. LASIK eye surgery. Updated January 27, 2026. Available at: https://medlineplus.gov/ency/article/007018.htm
5	Assign REF ID	American Academy of Ophthalmology. LASIK — Laser Eye Surgery. Reviewed by Hura AS. Edited by Turbert D. Available at: https://www.aao.org/eye-health/treatments/lasik
6	Assign REF ID	American Academy of Ophthalmology. Laser Surgery Recovery. Reviewed by Pagan-Duran B. Edited by Turbert D. Published May 25, 2023. Available at: https://www.aao.org/eye-health/treatments/laser-surgery-recovery
7	Assign REF ID	Mayo Clinic. LASIK eye surgery. Available at: https://www.mayoclinic.org/tests-procedures/lasik-eye-surgery/about/pac-20384774 . Related: Mayo Clinic. LASIK surgery: Is it right for you? Available at: https://www.mayoclinic.org/tests-procedures/lasik-eye-surgery/in-depth/lasik-surgery/art-20045751

8	Assign REF ID	Wang C, Wang X, Jin L, Tang B, Zhu W, Zhang G, Chen T, McAneney H, Kassalow J, Congdon N. Influence of presbyopia on smartphone usage among Chinese adults: a population study. Clin Exp Ophthalmol. 2019;47(7):909-917. doi:10.1111/ceo.13559
9	Assign REF ID	Courtin R, Saad A, Grise-Dulac A, Guilbert E, Gatinel D. Changes to corneal aberrations and vision after monovision in patients with hyperopia after using a customized aspheric ablation profile to increase corneal asphericity (Q-factor). J Refract Surg. 2016;32(11):734-741. doi:10.3928/1081597X-20160810-01

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[WATCHOUT]**WATCHOUT: THIS IS A GLOBAL TEMPLATE.**

IN ACCORDANCE WITH THE ALCON GLOBAL POLICY ON PROFESSIONAL PRACTICES (THE LENS) AND APPLICABLE LAWS, REGULATIONS, LOCAL INDUSTRY CODES AND LOCAL POLICIES (E.G., IMPORTANT PRODUCT INFORMATION (IPI)), THIS DOCUMENT MUST BE REVIEWED AND APPROVED VIA A LOCAL MATERIAL APPROVAL PROCESS (LMAP) BEFORE IT MAY BE USED IN PROMOTIONAL ACTIVITIES.

Appendix — Claim-by-claim support (original three PDFs + additional sources 4–9)

This grid is the working annotation for medical/legal review. Green = the cited source(s) state the same meaning. Amber = related support, but the copy is broader than the source or is a product-framed interpretation. Sources 4–9 were added to cover copy that the original three PDFs did not support. Quotes are taken from the cited sources. Procedure, comfort, and recovery sources describe LASIK generally, not READ™-specific outcomes.

Section / Copy	Copy excerpt	Ref(s)	Status	Supporting excerpt from source PDF
[Header] Subhead	Learn more about READ™, presbyopia laser vision correction	2, 3	Partial	2: LASIK, PRK, and SMILE “correct presbyopia by using monovision.” 3: “modifying the corneal asphericity by creating a hyperprolate corneal surface has been proposed as a surgical approach to compensate for presbyopia.” READ™ is not named.
Is My Vision Changing?	Difficulty with close-up tasks (reading a book, checking your phone, looking at a restaurant menu) may be a sign of presbyopia	1, 2	Supported	1: “Presbyopia is a refractive error that makes it hard for middle-aged and older adults to see things up close.” 2: “Have you noticed you’re holding your phone farther away... squinting to read that text? Having trouble with up-close tasks... you may be experiencing... presbyopia.” 2 also: “you’ll be reading menus again.”
Is My Vision Changing?	Presbyopia is a normal part of aging	1, 2	Supported	1: “Presbyopia is a normal part of aging. Everyone gets presbyopia as they get older — usually after age 45.” 2: “Presbyopia is part of the natural aging process of the eye... It’s not a disease, it’s as normal as wrinkles.”

Is My Vision Changing?	The natural lens becomes less flexible and stops focusing light correctly on the retina, which makes nearby objects look blurry	1, 2	Supported	1 (near-verbatim): "As you age, the lens in your eye gets harder and less flexible, and it stops focusing light correctly on the retina. This makes nearby objects look blurry." 2: "Presbyopia occurs when the aging lens of your eye becomes less flexible and can no longer focus on objects up-close... This causes close items to blur."
Common symptoms	Blurred vision up close	1, 2	Supported	1: "Trouble seeing things up close." 2: "Blurred vision at a normal reading distance."
Common symptoms	Holding reading material farther away to see it clearly	1, 2	Supported	1: "Needing to hold reading materials farther away to focus on them." 2: "The need to hold reading material at arm's length."
Common symptoms	Tired or strained eyes during close-up tasks	1, 2	Supported	1: "Eye strain (when your eyes feel tired or sore)." 2: "Eye strain." Also 2: "Headaches from doing close work."
Common symptoms	Headaches after reading or other close-up work	1, 2	Supported	1: Symptom list includes "Headache." 2: "Headaches from doing close work."
How is Presbyopia Managed?	Your eye care professional may suggest options to manage your vision	1, 2	Supported	1: As presbyopia gets worse, "you'll probably need glasses or contact lenses." 2: "Discuss the best choice for you with your eye care provider. Depending upon your overall health and lifestyle, your provider may suggest any of the following..."
Management bullets	Reading or prescription glasses	1, 2	Supported	1: OTC reading glasses or prescribed lenses. 2: "prescription glasses... reading glasses."
Management bullets	Progressive lenses	2	Supported	2: "progressive addition lenses" / "Progressives are multifocal lenses, similar to bifocals, but have a more gradual shift between the prescriptions."
Management bullets	Contact lenses	1, 2	Supported	1: "glasses or contact lenses to help you read." 2: bifocal, multifocal, monovision, and modified monovision contact lenses.
Management bullets	Lens replacement procedures	2	Supported	2: "Lens replacement: Some people are better suited for procedures that remove the natural lens... refractive lens exchange (RLE)."
Management bullets	Laser vision correction	2, 3	Supported	2: LASIK, PRK, SMILE listed as laser procedures that correct presbyopia (via monovision). 3: excimer laser keratorefractive surgery for "presbyopic compensation"; Custom Q (Alcon) Q-factor-optimized ablation profiles.
How is Presbyopia Managed?	A modern approach... READ™... may be recommended for presbyopia	2, 3	Partial	Laser for presbyopia is supported (2, 3). READ™ as a named procedure is not in the PDFs. 3 discusses Alcon Custom Q aspheric treatments that can plan induction of spherical aberration and are relevant to presbyopia compensation.

What is READ™?	Designed to address presbyopia	3	Partial	3: “modifying the corneal asphericity by creating a hyperprolate corneal surface has been proposed as a surgical approach to compensate for presbyopia.” Also: implications “during excimer laser keratorefractive surgery for both emmetropization and presbyopic compensation.”
What is READ™?	Optimize each eye so they can continue working together across near, intermediate, and far distances	2, 3	Partial	2: monovision (one eye distance, one near); modified monovision; multifocal lenses “include more than two focal points, including the intermediate zone of about 3 feet.” 3: induction of negative spherical aberration “may also serve to extend the depth of focus and through-focus visual acuity,” allowing a target to move closer with relatively small decrease of visual quality. Binocular “working together” is inferred from 2, not stated for READ™.
What is READ™?	With less dependence on glasses or contact lenses	2	Partial	2 presents refractive surgery as an alternative to glasses and contacts so patients can “see things up close again,” and notes that if glasses are used only part of the time, surgery may be more risk than reward. No spectacle-independence rate is given.
Could READ™ Be Right for Me?	Get back to the activities you love / read with ease / passions and hobbies	2	Partial	2 closing note: once you choose a correction option, “you’ll be able to focus again on all your favorite things in fine detail — whether it’s reading the latest novel, painting... or sewing.” Also: “you’ll be reading menus again in no time.”
Could READ™ Be Right for Me?	Stay connected and scroll through your day with clarity	5, 8, 9	Partial	5: LASIK “allows people to do most of their everyday tasks without corrective lenses.” 8: among smartphone-owning presbyopes, 64.3% reported difficulty using phones; difficulty was associated with worse near visual acuity. 9: after Custom-Q aspheric LASIK with monovision, 83% had J3 or better binocular uncorrected near acuity. The “scroll with clarity” phrasing is still promotional.
Could READ™ Be Right for Me?	Greater freedom from glasses and reclaim the freedom of natural vision	2	Partial	2 frames surgery/contacts/glasses as options to restore near focus. “Freedom of natural vision” is promotional and not sourced. Cite 2 only for the glasses-alternative meaning.
Could READ™ Be Right for Me?	READ™ may be an option if you have presbyopia and are interested in reducing dependence on glasses	2, 3	Partial	2: discuss with provider whether surgery is right; laser/lens options exist for presbyopia. 3: aspheric laser approach proposed to compensate for presbyopia. Candidacy for READ™ specifically is not in the PDFs.

FAQ 1	Eye exam, health history and lifestyle to confirm if you're a candidate	1, 2	Supported	1: "Eye doctors can check for presbyopia as part of a comprehensive eye exam." 2: diagnosis by thorough eye exam; before surgery, "Discuss your eye health, family history and lifestyle with your provider before you decide if surgery is right for you, and which option fits your lifestyle best."
FAQ 1	The procedure is quick, usually a few minutes per eye	4, 7	Partial	4: "It takes 10 to 15 minutes to perform for each eye." 7: "LASIK surgery is usually completed in 30 minutes or less." Supports that the procedure is short/outpatient. "A few minutes per eye" is a looser paraphrase of 10–15 minutes.
FAQ 2	READ™ can reduce your dependence on glasses or contact lenses	2	Partial	Same support as the glasses-dependence claim above. No READ™ outcomes data in the three PDFs. "Individual results may vary" is uncited disclaimer language.
FAQ 3	Numbing eye drops; little to no discomfort; postoperative medication/drops for temporary discomfort	4, 5, 7	Supported	4: "The only anesthetic used is eye drops that numb the surface of your eye." Surgeon may prescribe mild pain medicine; postoperative drops to prevent infection and inflammation. 5: "Your eye will be numbed with eye drops." Afterward, eyes may feel scratchy/burning; "special eye drops to reduce dryness and help your eye heal." 7: numbing drops; "You generally will experience little pain"; "pain medicine or eye drops to keep you comfortable for several hours after the procedure."
FAQ 4	Many people return to normal activities the next day; temporary side effects; recovery differs	4, 6, 7	Supported	6: "In most cases, you can return to work, as well as drive, the day after the procedure." Temporary blurry/changing vision, dry eyes, glare or halos can last weeks or months; PRK takes longer. 4: blurriness improves by the next day; some people take days to 3–6 months to stabilize. 7: "vision recovery usually takes place in 1 to 2 days"; temporary side effects (dry eyes, vision changes) are fairly common.
[General Note]	Educational only; not a substitute for professional judgment; consult your eye care professional	1	Partial	1: "The National Eye Institute does not diagnose diseases, offer specific treatment recommendations, or provide physician referrals." Supports the educational-not-advice meaning; Alcon legal wording is its own.
[CTA]	Talk with your eye care professional to see if READ™ is right for you	2	Supported	2: "Discuss the best choice for you with your eye care provider." and "Discuss your eye health, family history and lifestyle with your provider before you decide if surgery is right for you."

Source snapshot (what each PDF is good for)

- REF-29185 (1) — NIH patient page. Best support for: definition of presbyopia, "normal part of aging," lens flexibility / light not focusing on the retina / near blur, symptom list, glasses and contact lenses, comprehensive eye exam, educational disclaimer tone. Does not cover surgery, laser, READ™, recovery, or comfort.

- REF-15633 (2) — Cleveland Clinic patient page. Best support for: near-task examples (phone, menus), aging lens, symptoms, the management menu (readers, prescription glasses, progressives, contacts, laser, lens replacement), candidacy conversation (exam, health history, lifestyle), and returning to close-up activities. Laser for presbyopia is described as monovision LASIK/PRK/SMILE, not as READ™.
- REF-20457 (3) — Gatinel et al., Journal of Refractive Surgery, 2014. Best support for: aspheric / Q-value customized excimer ablation (including Alcon Custom Q) to induce negative spherical aberration, extend depth of focus, and compensate for presbyopia. Does not name READ™ and does not contain patient-facing outcome, comfort, or recovery data.
- MedlinePlus (4) — LASIK encyclopedia page. Best support for: 10–15 minutes per eye; topical anesthetic (numbing) drops only; postoperative pain medicine and anti-inflammatory/antibiotic drops; vision often clearer the next day; recovery varies.
- AAO LASIK (5) — Patient page. Best support for: numbing drops during surgery; postoperative lubricating/healing drops; everyday tasks without glasses/contacts; temporary side effects (scratchiness, dry eye, glare).
- AAO Laser Surgery Recovery (6) — Best support for: most people return to work and driving the day after LASIK; temporary side effects; PRK recovers more slowly.
- Mayo Clinic (7) — Best support for: procedure usually ≤30 minutes; numbing drops; little pain; postoperative pain medicine or drops; vision recovery in 1–2 days.
- Wang et al. (8) — Population study. Best support for: uncorrected presbyopia commonly makes smartphone use difficult (64.3% of smartphone-owning presbyopes); worse near acuity → more difficulty.
- Courtin et al. (9) — Custom-Q clinical series (Alcon Custom-Q / WaveLight; READ-related method). Best support for: uncorrected near acuity after aspheric presbyopic LASIK (83% J3 or better binocular UNVA at 6 months).

[Logo] Alcon